NCCP 00849 · clinician draft

# Nivolumab, gemcitabine and cisplatin before surgery

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 1 · gemcitabine / cisplatin with nivolumab

**Active medicines:** nivolumab + gemcitabine + cisplatin

**Emetogenic risk:** high

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally Day 1; 80 mg orally Days 2–3 | Before SACT on Day 1, then daily on Days 2–3 |
| Ondansetron | 16 mg orally | Day 1<br>Adjust for QT/individual risk. |
| Antiemetic dexamethasone reference | 12 mg orally Day 1; 8 mg orally daily Days 2–4 | General high-risk NCIS V8 row<br>Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription. |
| Olanzapine | 5 mg orally once daily | Day 1 and Days 2–4<br>General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise. |
| Metoclopramide | 10 mg orally three times daily as prescribed | Subsequent days<br>NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions. |

### Additional phase-specific support

- Follow the written anti-sickness plan; Day 8 gemcitabine has its own review and support instructions. Cisplatin needs hydration, electrolyte checks and monitoring of kidneys, hearing and nerves.
- Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose.
- The prescriber specifies doses, duration, rescue treatment and interactions; follow one reconciled prescription.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00849 v2 · pNCCP 00849 pp1–7; NCCP 00281 supportive care; NCIS V8 Table 1 · NCCP NCIS antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required